Group A

What is the most common cause of serious upper GI bleeding?

A. Esophageal varices
B. Erosive gastritis
C. Peptic ulcer disease
D. Mallory-Weiss tear

What is the mechanism of death in postpartum acute gastric dilatation?

A. Pulmonary embolism
B. Aspiration of gastric contents
C. Acute renal failure
D. Ischemia and necrosis of the stomach

Which gastric ulcer is most liable to be associated with stomach cancer?

A. Gastric ulcer associated with atrophic gastritis
B. Duodenal ulcer without gastric inflammation
C. Gastric ulcer caused by acute stress
D. Gastric ulcer with normal surrounding mucosa

Group B

What is a Richter's hernia?

A. Herniation of the entire circumference of the bowel
B. Herniation of anti-mesenteric border of the bowel
C. Herniation of the mesenteric border only
D. Herniation of the stomach through the diaphragm

A patient with suspected celiac disease has normal total IgA and is still eating gluten. Which initial antibody test is generally preferred?

A. tTG-IgA
B. Anti-centromere antibody
C. IgG antimitochondrial antibody
D. IgM rheumatoid factor

What is the most common indication for surgery in peptic ulcer disease?

A. Uncomplicated heartburn
B. Perforated ulcer
C. Asymptomatic H. pylori infection
D. Recurrent bleeding ulcer

Barrett esophagus predisposes to which type of cancer?

A. Gastrointestinal stromal tumor
B. Adenocarcinoma
C. Small-cell carcinoma
D. Squamous cell carcinoma

Group C

Which diet is associated with diverticulosis?

A. Low residue diet
B. High-protein diet
C. High-fat diet
D. High-residue diet

Which finding indicates shock in a patient with GI bleeding?

A. BP 110/70 mmHg with pulse 80 bpm
B. BP 140/90 mmHg with pulse 60 bpm
C. BP 90/60 mmHg with pulse 110 bpm
D. BP 130/80 mmHg with pulse 75 bpm

Which statement about stomach cancer is correct?

A. Barium meal is the definitive staging test
B. Serum CEA alone confirms the diagnosis and stage
C. Ultrasound is a basic tool important in staging
D. CT cannot detect distant metastases

What is the best management for esophageal cancer with liver metastasis?

A. Curative esophagectomy
B. Definitive chemoradiotherapy with curative intent
C. Palliative stenting and supportive care
D. Endoscopic mucosal resection

What can cause exudative diarrhea?

A. Rapid intestinal transit
B. Unabsorbed lactose
C. Excessive water intake
D. Ulceration of the mucosa

Group D

Which noninvasive test relies on bacterial conversion of labeled urea to labeled carbon dioxide?

A. Fecal elastase test
B. Urea breath test
C. Hydrogen breath test for lactose
D. Serum calcium test

Esophageal squamous cell carcinoma is associated with which factors?

A. H. pylori and celiac disease
B. Obesity and Barrett's esophagus
C. Lactose intolerance and smoking
D. Alcohol and smoking

What is the best definition of constipation?

A. Urgency with fecal incontinence
B. Infrequent bowel movements with difficulty passing stool
C. Daily soft stools without straining
D. Frequent loose stools with urgency

Group E

What is the most important factor for incisional hernia recurrence?

A. Large defect size
B. Duration of preoperative fasting
C. Patient's blood group
D. Time of day of surgery

Untreated chronic reflux can lead to which structural complication?

A. Appendiceal abscess
B. Esophageal stricture
C. Ileal atresia
D. Femoral hernia

What is the most common type of hiatus hernia?

A. Mixed type hiatus hernia
B. Sliding hiatus hernia
C. Type IV hiatus hernia
D. Paraesophageal hiatus hernia

What is melena?

A. Passage of pale stools due to bile duct obstruction
B. Passage of black tarry stools due to altered blood in the gut
C. Passage of fresh red blood from the rectum
D. Passage of fatty stools due to malabsorption

Group F

Chronic diarrhea is clinically defined as symptoms lasting more than how many days?

A. 7 days
B. 30 days
C. 14 days
D. 90 days

The cancer associated with a small proportion of Barrett's esophagus cases is most often which histologic type?

A. Cutaneous melanoma
B. Esophageal adenocarcinoma
C. Renal cell carcinoma
D. Hepatocellular carcinoma

Which finding makes esophageal carcinoma unresectable?

A. Mild mucosal erythema
B. Tracheobronchial invasion
C. A tumor confined to the mucosa
D. A small superficial lesion

Which pattern of Crohn's disease is most common?

A. Disease in the ileum and cecum
B. Disease limited to the rectum
C. Isolated disease of the esophagus
D. Disease limited to the stomach

Which statement about bleeding from Meckel's diverticulum is true?

A. It is caused by variceal rupture
B. It indicates ulceration of the adjacent segment of ileum
C. It indicates a bleeding gastric polyp
D. It is due to appendiceal inflammation

Group G

Which laboratory finding distinguishes bacterial overgrowth from tropical sprue?

A. Low serum folate with high B12
B. Normal or high serum folate with low B12
C. High serum folate and high B12
D. Low serum folate and low B12

An inflammatory bowel condition is limited to the colon and usually affects its inner lining. Which condition fits this pattern?

A. Achalasia
B. Lactase deficiency
C. Crohn's disease of the mouth
D. Ulcerative colitis

A patient has epigastric pain, antral erythema, and chronic inflammation. What is the most likely cause?

A. Cytomegalovirus infection
B. Helicobacter pylori infection
C. Autoimmune gastritis
D. Acute pancreatitis

Group H

What causes diarrhea in lactase deficiency?

A. Osmotic effect of unabsorbed lactose
B. Increased intestinal secretion of chloride
C. Reduced bile acid production
D. Ulceration of the intestinal mucosa

Which is an alarm feature in constipation?

A. Iron-deficiency anemia
B. Occasional bloating
C. Hard stools
D. Straining during defecation

What is the surgical approach for duodenal ulcer bleeding that is not controlled endoscopically?

A. Nissen fundoplication
B. Wedge resection of the gastric fundus
C. Exploration and over-sewing of the bleeding ulcer base
D. Vagotomy alone without controlling the bleeding vessel

Colonoscopy reveals separated areas of inflammation with normal-appearing intervening mucosa. Which disease pattern is most consistent?

A. Continuous ulcerative colitis only
B. Simple reflux
C. Lactose malabsorption
D. Patchy Crohn's disease

Group I

A 28-year-old woman presents with bloody diarrhea and continuous rectal inflammation. What is the most likely diagnosis?

A. Crohn's disease
B. Ischemic colitis
C. Ulcerative colitis
D. Infectious colitis

A patient's pyloric obstruction has not responded to dilation. Which statement about management is correct?

A. Perform colectomy
B. Continue dilation indefinitely without reassessment
C. Faulty decision → patient needs vagotomy and gastrojejunostomy
D. Perform fundoplication alone

Which investigation is the choice for diagnosing and treating upper GI disorders?

A. Abdominal CT scan
B. Barium swallow
C. Esophagogastroduodenoscopy (EGD)
D. Esophageal manometry

Recurrent abdominal pain associated with altered bowel habits occurs without visible structural damage. What broad disorder category fits?

A. A mandatory malignancy diagnosis
B. Mechanical obstruction in every case
C. Disorder of gut–brain interaction
D. A primary kidney disorder

Which type of colonic polyp may become malignant?

A. Hyperplastic polyp
B. Adenomatous polyp
C. Juvenile hamartomatous polyp
D. Inflammatory polyp

Group J

A male patient presents with severe epigastric distension and vomiting and has a history of bulimia nervosa. What is the expected diagnosis?

A. Superior mesenteric artery syndrome
B. Distal pyloric obstruction on top of ulcer
C. Gastric volvulus
D. Acute pancreatitis due to recurrent vomiting

What is the characteristic histology of celiac disease?

A. Villous hypertrophy with crypt atrophy
B. Villous atrophy with crypt hyperplasia
C. Normal villi with increased gastric pits
D. Granulomas without villous changes

For bleeding thought to arise in the small intestine, which swallowed device records images as it passes through the gut?

A. Nasogastric suction tube
B. Electrocardiograph lead
C. Rectal thermometer
D. Capsule endoscope

Group K

Which is the most specific serology for celiac disease?

A. IgG anti-mitochondrial antibody
B. IgA anti-gliadin antibody
C. IgM anti-endomysial antibody
D. IgA anti-transglutaminase antibody

Achalasia predisposes to which type of cancer?

A. Adenocarcinoma
B. Gastrointestinal stromal tumor
C. Small-cell carcinoma
D. Squamous cell carcinoma

An untreated inflamed appendix can result in which complication?

A. Primary achalasia
B. Peritonitis or an abscess
C. Lactase persistence
D. Barrett's esophagus

What is the dominant symptom in ulcerative colitis?

A. Constipation
B. Painless jaundice
C. Steatorrhea
D. Bloody diarrhea

Group L

What percentage of fluid is absorbed in the small intestine?

A. 99%
B. 50%
C. 10%
D. 90%

60-yo, 4-year intermittent dysphagia, CXR: absent fundic air bubble. TRUE:

A. Esophageal motility disorder highly suspected
B. pH study highly indicated
C. Surgery is first line
D. Endoscopy commonly reveals tight stricture

The D-xylose test is used for:

A. Diagnosing C. difficile
B. Diagnosing acute infectious diarrhea
C. Differentiating small-bowel from pancreatic malabsorption
D. Testing gastric acid secretion

The most common position of the appendix is:

A. Pelvic
B. Pre-ileal
C. Sub-hepatic
D. Retrocecal (~65%)

Which autoantibody is found in celiac disease?

A. Anti-endomysial / anti-tTG
B. ANCA
C. ASCA
D. ANA

Group M

In malabsorption, which test evaluates ileal function (bile acid absorption)?

A. Hydrogen breath test
B. SeHCAT (selenium-homotaurocholic acid) test
C. Fecal elastase
D. Secretin stimulation test

Barrett oesophagus is predisposed to:

A. Leiomyosarcoma
B. Squamous cell carcinoma
C. Lymphoma
D. Adenocarcinoma

47-y-old woman with chronic diarrhea + bloating (worse with wheat) + lethargy + weight loss + anemia. Most likely diagnosis?

A. Lactose intolerance
B. Ulcerative colitis
C. Celiac disease
D. Crohn's disease

Group N

Which autoantibody is found in celiac disease?

A. ANA
B. ANCA
C. EMA (anti-endomysial Ab)
D. ASCA

Obese 44-yo severe heartburn — best tool for hiatus hernia:

A. CT upper abdomen
B. Esophageal motility
C. Abdominal US
D. Barium meal in Anti-Trendelenburg (head-down)

Which diagnostic test shows the cobblestone appearance and aphthous ulcers in Crohn's disease?

A. Abdominal ultrasound
B. MRI head
C. Barium follow-through / small-bowel enema
D. Abdominal CT

Which is CORRECT regarding tropical sprue?

A. Involves only proximal small bowel
B. No resemblance to celiac disease
C. Immune-mediated disease
D. Caused by an infectious (bacterial) etiology

Group O

In malabsorption, which test assesses ileal function?

A. Hydrogen breath test
B. SeHCAT (bile acid absorption)
C. Secretin stimulation
D. Fecal elastase

A 26-year-old has had eight months of crampy abdominal pain, diarrhea, weight loss, aphthous ulcers, conjunctivitis, and right lower quadrant tenderness. What is the most likely diagnosis?

A. Celiac disease
B. Lactose intolerance
C. Crohn disease
D. Ulcerative colitis

In Crohn's, B12 deficiency occurs from involvement of:

A. Proximal ileum
B. Terminal ileum
C. Jejunum
D. Colon

A 47-year-old has chronic diarrhea and steatorrhea after eating wheat, fatigue, and iron-deficiency anemia. What is the most likely diagnosis?

A. Crohn disease
B. Ulcerative colitis
C. Celiac disease
D. Lactose intolerance

Which test diagnoses bacterial overgrowth as a cause of malabsorption?

A. Hydrogen breath test (glucose/lactulose)
B. Fecal elastase
C. Schilling test
D. D-xylose test

Group P

Which part of the small bowel is responsible for vitamin B12 absorption?

A. Jejunum
B. Cecum
C. Terminal ileum
D. Duodenum

Infant with CHPS — expected US finding:

A. Small cystic mass at pyloric area
B. Absent pylorus
C. Pyloric channel length elongated (>16 mm)
D. Pyloric wall thickness < 3 mm

Bloody diarrhea can occur in all of the following EXCEPT:

A. Ischemic colitis
B. Diverticulitis
C. Celiac disease
D. C. difficile infection

Group Q

The following is a characteristic finding in malignant pyloric channel carcinoma:

A. CA 72-4 has higher specificity in this type of cancer
B. Epigastric pain radiates to the back
C. GIST is the most common type
D. The stomach shadow in a barium meal usually reaches the pelvis

---

Which statement about tropical sprue is correct?

A. Immune-mediated disease
B. No resemblance to celiac disease
C. Responds to prolonged antibiotics + folate/B12
D. Involves only proximal small bowel

D-xylose absorption test is used for:

A. Diagnosing acute infectious diarrhea
B. Differentiating small bowel vs hepatic causes of malabsorption
C. Diagnosing C. difficile
D. Differentiating small bowel (mucosal) vs pancreatic causes of malabsorption

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A 19-year-old has chronic diarrhea, bloating, and weight loss that worsen with wheat, along with iron-deficiency anemia and flattened villi on duodenal biopsy. Which specific diagnostic test is indicated?

A. Anti-tissue transglutaminase (anti-tTG) IgA
B. Schilling test
C. Secretin test
D. D-xylose absorption test

Group R

Which test is used to diagnose bacterial overgrowth as a cause of malabsorption?

A. D-xylose test
B. Hydrogen breath test
C. Schilling test
D. Fecal elastase

Which food group is specifically listed among high-FODMAP foods to limit in the GI book?

A. Plain water
B. Onions and garlic
C. Egg whites
D. Lean poultry

Which statement best reflects how IBS is diagnosed as a disorder of gut–brain interaction?

A. By endoscopy in every patient
B. By elevated liver enzymes
C. Primarily by clinical criteria such as Rome guidelines when routine tests are usually normal
D. By a specific structural lesion on imaging

Mucus and blood leaking from an actively inflamed intestinal mucosa are characteristic of which diarrhea mechanism?

A. Secretory
B. Exudative
C. Osmotic
D. Altered motility

Which factor can alter gut motility and processing through the gut–brain axis in IBS?

A. Portal hypertension only
B. A fixed intestinal stricture
C. Gallstones only
D. Stress and anxiety

Group S

Which is the normal range of stool frequency stated in the GI book?

A. Four to six times daily
B. Three times a week to three times a day
C. Once every two weeks to twice a week
D. Once daily only

Diarrhea lasting more than four weeks should be classified and evaluated as:

A. Acute diarrhea
B. Chronic diarrhea
C. Food poisoning only
D. A normal variation

Which ion is the major actively absorbed ion in the intestine according to the GI book?

A. Bicarbonate
B. Potassium
C. Sodium
D. Chloride

Group T

After negative gastroscopy and colonoscopy, which test may be used to investigate obscure GI bleeding from the small bowel?

A. ERCP
B. Liver biopsy
C. Wireless capsule endoscopy
D. Plain abdominal X-ray

Which statement about probiotics for IBS matches the GI book?

A. They are diagnostic rather than therapeutic
B. There is no routine recommendation for probiotics
C. They replace all dietary measures
D. They are routinely recommended for every patient

Approximately what proportion of acute diarrhea is attributed to infection in the GI book?

A. 30%
B. 10%
C. 80%
D. Nearly 100%

Which IBS subtype is characterized by predominantly loose stools?

A. IBS-D
B. IBS-C
C. IBS-U
D. IBS-M

Group U

With low dietary fibre intake, which stool output is listed as meeting the GI book's diarrhea definition?

A. More than 1 kg per day
B. More than 50 g per day
C. More than 100 g per day
D. More than 200 g per day

IBS with both constipation and diarrhea is classified as:

A. IBS-U
B. IBS-C
C. IBS-M
D. IBS-D

Which statement about anticoagulants and acute GI bleeding is most accurate?

A. They do not usually cause bleeding by themselves, but can worsen bleeding from another lesion
B. They commonly cause ulcers directly
C. They only increase lower GI bleeding
D. They prevent bleeding from peptic ulcers

Early gastric carcinoma is defined by invasion limited to which layers?

A. Mucosa and submucosa
B. Serosa only
C. Any depth if there are no symptoms
D. Muscularis propria only

Which electrolyte disturbance is listed as a risk factor for functional acute gastric dilatation?

A. Hypercalcemia only
B. Severe hypokalemia
C. Hypophosphatemia only
D. Isolated hypernatremia

Group V

The core clinical presentation of irritable bowel syndrome is:

A. Persistent jaundice
B. Recurrent abdominal pain related to defecation with altered bowel habits
C. Painless rectal bleeding
D. Progressive dysphagia

Which stool test is especially useful as a marker of intestinal inflammation in suspected IBD?

A. Fecal occult fat
B. Fecal elastase
C. Fecal calprotectin
D. Stool pH

Which finding may indicate that conservative management of acute gastric dilatation is succeeding?

A. Pneumoperitoneum on CT
B. Persistent severe pain after NG decompression
C. New peritonitis
D. Pain resolves after NG decompression and imaging shows no ischemia or perforation

Group W

Which symptom is an alarm feature that warrants investigation in a patient with dyspepsia?

A. Progressive dysphagia
B. Intermittent flatulence
C. Belching
D. Occasional bloating

Which finding is an indication for emergency laparotomy in acute gastric dilatation?

A. Failure of NG decompression
B. Successful NG decompression with pain relief
C. Mild isolated bloating
D. Normal gastric wall enhancement

Which dietary approach is described in the GI book for managing IBS symptoms?

A. Exclusive parenteral nutrition
B. Low-FODMAP diet
C. Lifelong gluten elimination for every patient
D. High-FODMAP diet

The most common gross type of esophageal carcinoma described in the GI book is:

A. Exophytic
B. Scirrhous
C. Multicystic
D. Flat infiltrating

Group X

How much fluid is delivered to the small intestine daily on a normal diet, according to the GI book?

A. About 1 L
B. About 3 L
C. About 20 L
D. About 9 L

A villous adenoma of the colon is typically:

A. A non-neoplastic inflammatory lesion
B. Multiple and pedunculated
C. Single, large, sessile, and broad-based
D. Always smaller than a tubular adenoma

Which statement about fluid absorption in the small intestine is correct?

A. No fluid is absorbed before the colon
B. The small intestine normally absorbs only 10%
C. All fluid is excreted in stool
D. About 90% of the delivered fluid is absorbed there

Which gastric polyp type has essentially no carcinoma risk in the GI book?

A. Fundic gland polyp
B. Hyperplastic polyp
C. Villous polyp
D. Adenomatous polyp

In acute gastric dilatation, contrast CT is especially useful to assess gastric dimensions and:

A. Portal vein pressure
B. Wall enhancement and the position of the NG tube
C. Appendiceal diameter
D. Gallbladder ejection fraction

Group Y

Difficulty swallowing is termed:

A. Dyspepsia
B. Odynophagia
C. Dysphagia
D. Regurgitation

A serious complication of acute gastric dilatation is:

A. Gastric perforation with hemodynamic collapse
B. Esophageal varices
C. Painless jaundice
D. Appendiceal rupture

Which mechanism best describes secretory diarrhea?

A. Mechanical bowel obstruction
B. Loss of pancreatic lipase
C. Increased intestinal chloride and water secretion
D. Unabsorbed solutes remaining in the lumen

Group Z

The first supportive step for a patient with acute gastric dilatation is:

A. Immediate oral feeding
B. Elective colonoscopy
C. Long-term antacids alone
D. Nasogastric decompression and IV fluid resuscitation

Compared with acute diarrhea, most causes of chronic diarrhea are:

A. Always parasitic
B. Due to viruses only
C. Noninfectious
D. Due to food allergy only

What is the primary mechanism of osmotic diarrhea?

A. Reduced colonic motility
B. Active chloride secretion
C. Mucosal bleeding
D. Poorly absorbed osmotically active solutes in the bowel lumen

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Treatment of early intussusception in children:

A. Antibiotics only
B. Immediate surgical resection always
C. IV fluids only
D. Conservative pneumatic/hydrostatic enema reduction (success rate >80% when treated early)

Group AA

Why is acute appendicitis in elderly and children usually complicated?

A. The appendix is always retrocecal in them
B. The omentum and peritoneal cavity are less efficient in localizing the disease
C. Antibiotic response is poor in these ages
D. The appendix is less vascular in these age groups
E. They tolerate pain better and present late only

On an erect plain X-ray, air under the diaphragm indicates:

A. Normal
B. Ileus
C. Perforated viscus (pneumoperitoneum) → surgical exploration
D. Simple SBO

When assessing intestinal obstruction, what is the correct sequence?

A. No need to distinguish between types
B. CT, followed by anything else
C. Mechanical vs functional → cause → simple vs complicated → management
D. Immediate surgery

Which is NOT a barium enema finding in Ulcerative Colitis?

A. Loss of haustrations (lead-pipe colon)
B. Shortened colon
C. Granular mucosa
D. Skip lesions with cobblestoning
E. Pseudopolyps

In acute intestinal obstruction, signs of peritonitis indicate:

A. Sigmoid volvulus resolved
B. Strangulation
C. Pseudo-obstruction
D. Paralytic ileus
E. Simple obstruction

Group AB

Infantile intussusception usually occurs during:

A. 2–5 years
B. First year of life (esp. 6–12 months)
C. First month of life
D. Adolescence
E. School age

A 25-year-old man with recurrent discharging peri-anal opening, weight loss, bloody diarrhea, tenesmus, and normal-looking rectum on proctoscopy. Most likely diagnosis:

A. Cryptoglandular anal fistula
B. Crohn's disease
C. Ulcerative colitis
D. TB proctitis
E. Rectal cancer

Which is NOT a feature of generalized peritonitis?

A. Board-like abdomen
B. Silent abdomen on auscultation
C. Severe colicky abdominal pain
D. Diffuse tenderness and rigidity
E. Fever and tachycardia

Group AC

The nature of pain in small-intestinal obstruction:

A. Constant dull ache
B. Colicky, sharp, periumbilical pain that coincides with peristaltic waves
C. Burning epigastric
D. Radiating to back

A corroborative sign of acute appendicitis is:

A. Rovsing's sign — RIF pain on LIF pressure
B. Cullen's sign
C. Relief of pain with thigh extension
D. Relief of pain with internal rotation of the right thigh
E. Decrease of pain with testicular elevation

Indication for total proctocolectomy in Ulcerative Colitis includes all EXCEPT:

A. Toxic megacolon not responding to medical therapy
B. Occasional bouts of colic and diarrhea
C. Evidence of malignancy/dysplasia
D. Massive uncontrolled hemorrhage
E. Failure of medical treatment

A child presents with colicky pain, a right iliac fossa mass, and red currant jelly stools:

A. Intussusception (ileocolic — the most common form in children)
B. Volvulus
C. Meckel's
D. Appendicitis

Group AD

The usual presentation of uncomplicated diverticulosis is:

A. Fever and throbbing pain
B. Bleeding per rectum
C. Absolute constipation
D. Asymptomatic
E. Severe LIF pain with rigidity

The commonest cause of intestinal obstruction in children is:

A. Paralytic ileus
B. Neoplasm
C. Intussusception
D. Strangulated hernia
E. Adhesions

The best investigation for uncomplicated diverticular disease is:

A. Angiography
B. Colonoscopy
C. Abdominal US
D. Double-contrast barium enema
E. Plain X-ray

A finding consistent with Ulcerative Colitis on colonoscopy is:

A. Cobblestoning and deep fissures
B. Transmural (full-thickness) inflammation
C. Terminal ileum involvement
D. Confluent disease with rectal involvement and no skip areas
E. Rectal sparing with skip areas

Investigation of choice for CHPS (congenital hypertrophic pyloric stenosis):

A. Abdominal ultrasound (pyloric muscle >3 mm, length >15 mm)
B. Upper endoscopy
C. Plain X-ray
D. Barium meal

Group AE

A 32-y-old with large-volume hematemesis. Most important FIRST step?

A. Nasogastric tube insertion
B. Fluid resuscitation (2 large-bore IV lines)
C. Full history and examination
D. Upper GI endoscopy

Regarding inguinal hernia and inferior epigastric vessels:

A. Both lateral
B. Both medial
C. Indirect is medial, Direct is lateral
D. Indirect is lateral, Direct is medial

The coverings of a true umbilical hernia sac in adults include:

A. None
B. Rectus abdominis alone
C. Skin + external oblique aponeurosis only
D. Skin, umbilical fascia, fascia transversalis, extraperitoneal fat, peritoneum

Group AF

An elderly woman with a 'benign' ulcer on greater curvature 5 cm proximal to antrum, still with anemia + guaiac-positive stool after 3 months of medical therapy. Next step?

A. Partial gastrectomy (suspect malignancy)
B. Total gastrectomy
C. Continue medical therapy
D. H. pylori eradication only

The following is NOT a cause of para-umbilical hernia recurrence:

A. Wound infection
B. Inlay (sublay) mesh repair
C. Uncontrolled ascites / ↑ IAP
D. Absorbable sutures

An 'incisura ulcer' of the stomach is classified as:

A. Type 2 (gastric + duodenal)
B. Type 3 (prepyloric)
C. Type 1 (lesser curve, normal acid)
D. Type 4 (near GE junction)

47-yo with bleeding DU, pulse 90, BP 100/70, failed medical + endoscopic control. Best surgical option:

A. Excision of ulcer alone
B. Total gastrectomy
C. Over-sewing of the bleeding vessel + truncal vagotomy + pyloroplasty
D. Continue transfusion only

Group AG

Causes of hernia recurrence include all EXCEPT:

A. Repair under tension
B. Tissue (herniorrhaphy) repair
C. Wound infection
D. Tension-free Lichtenstein mesh

One of the following sites NEVER develops diverticulosis:

A. Rectum
B. Sigmoid colon
C. Cecum
D. Transverse colon

Heller's cardiomyotomy is best performed by:

A. None
B. Laparoscopy (with partial fundoplication)
C. Upper endoscopy
D. Open thoracotomy

Why is appendicitis more dangerous in old age?

A. Fever is always high
B. Omental ability to encapsulate is reduced + rapid vascular thrombosis
C. Pain is severe
D. Appendix is absent

The structure most commonly responsible for strangulation of a femoral hernia is:

A. Pectineus
B. Inguinal ligament
C. Lacunar (Gimbernat's) ligament
D. Cooper's ligament

Group AH

Female on medical Tx for chronic DU develops GOO; endoscope can't pass duodenum. Best surgical option:

A. Vagotomy + pyloroplasty alone (usually inadequate for fibrotic stenosis)
B. Barium meal follow-up
C. Distal gastrectomy + gastro-jejunostomy (Billroth II) ± vagotomy
D. Endoscopic dilatation only

55-year-old on NSAIDs for arthritis develops severe epigastric pain with board-like rigidity; erect CXR shows air under the right dome of diaphragm. Diagnosis:

A. Ruptured hepatic abscess
B. Acute pancreatitis
C. Perforated peptic ulcer
D. Acute cholecystitis

Ivor Lewis operation refers to:

A. Laparotomy + right thoracotomy: gastric pull-up with intra-thoracic esophago-gastric anastomosis
B. POEM
C. Total gastrectomy
D. Transhiatal esophagectomy

Group AI

'Bubonocele' refers to an inguinal hernia that:

A. Contains bladder
B. Is irreducible
C. Is confined to the inguinal canal (does not pass external ring)
D. Extends into the scrotum

Billroth II operation means:

A. Highly selective vagotomy
B. Lower radical gastrectomy + gastro-duodenal anastomosis
C. Total gastrectomy + Roux-en-Y esophago-jejunostomy
D. Lower radical gastrectomy + gastro-jejunostomy

The lower third of the esophagus is supplied by:

A. Short gastric vessels
B. Right gastric vessels
C. Left gastro-epiploic vessels
D. Left gastric vessels

A hernia is called Littre's hernia when its contents include:

A. Meckel's diverticulum
B. Bladder wall
C. Ovary
D. Appendix

Group AJ

Patient with long ulcer dyspepsia vomits large amounts of old undigested food; epigastric distension. Diagnosis:

A. Cancer stomach fungating
B. Gastric outlet obstruction due to chronic DU (fibrotic pyloric stenosis)
C. Acute gastritis
D. Achalasia

The most accurate investigation to diagnose Meckel's diverticulum is:

A. CT abdomen
B. Angiography
C. Tc-99m pertechnetate (Meckel's) scan
D. Abdominal US

Sliding hernia does NOT occur in:

A. Bubonocele (small hernia within canal)
B. Elderly patients
C. Large hernias
D. Scrotal hernia

Barium swallow does NOT show:

A. Anatomical distortion
B. Mucosal irregularity
C. Regional lymph nodes in cancer
D. Luminal narrowing

Squamous cell carcinoma of the esophagus most commonly occurs in:

A. Upper thoracic esophagus
B. Cervical esophagus
C. Middle thoracic esophagus
D. Lower thoracic esophagus / GEJ

Group AK

Which is NOT a predisposing factor for esophageal cancer?

A. Achalasia
B. Columnar metaplasia (Barrett's)
C. Chronic reflux esophagitis
D. Long-standing esophageal varices

In esophageal cancer, investigation for mediastinal + LN invasion:

A. Bronchoscopy
B. Endoluminal ultrasonography (EUS)
C. Esophagoscopy
D. Barium swallow

Radiotherapy as 1st-line gives results similar to surgery in:

A. Pancreatic exocrine carcinoma
B. Barrett's adenocarcinoma
C. RCC
D. HCC
E. SCC of skin

Group AL

Which bariatric procedure induces MALABSORPTION?

A. Total gastrectomy
B. Sleeve gastrectomy
C. Roux-en-Y gastric bypass
D. Adjustable gastric band

All the following predispose to stress ulceration of the stomach EXCEPT:

A. Acute pancreatitis
B. Major burn (Curling's ulcer)
C. Deep vein thrombosis
D. Head injury (Cushing's ulcer)
E. Septic shock

The INCORRECT statement about secretory diarrhea is:

A. Diarrhea continues while fasting for 24–48 h
B. Lactose intolerance is a classic example
C. Stool volume is usually large (>1 L/day)
D. Stools do not contain pus or blood

The most common benign tumor of the esophagus is:

A. Lipoma
B. Hemangioma
C. Fibroma
D. Leiomyoma

Group AM

Features of ulcerative colitis include:

A. Mouth-to-anus distribution
B. Transmural inflammation
C. Marked pseudopolyps
D. Skip lesions

Which is associated with exudative diarrhea?

A. Celiac disease
B. Salmonella infection
C. Lactase deficiency
D. Vibrio cholerae

Indications for surgery in peptic ulcer include all EXCEPT:

A. Pyloric stenosis
B. Uncomplicated acute ulcer responsive to PPI
C. Perforation
D. Bleeding with failed endoscopic control

Morphological features of Crohn's disease include:

A. Broad-based ulcers
B. Skip lesions
C. Marked pseudopolyps
D. Left-sided disease

Which is NOT a relative indication for surgery in bleeding peptic ulcer?

A. Elderly patient
B. Recurrence of bleeding after endoscopic control
C. Long history of peptic ulcer
D. Patient with associated serious disease

Group AN

Regarding congenital hypertrophic pyloric stenosis, TRUE statement:

A. Presents in first 2 days of life
B. Bilious vomiting
C. Diagnosed by ultrasound
D. Metabolic acidosis
E. More common in females

Patients with chronic pyloric stenosis characteristically develop:

A. Respiratory alkalosis
B. Hyperkalemia and hypernatremia
C. Hypochloremic hypokalemic metabolic alkalosis
D. Metabolic acidosis with hyperkalemia

Surgery for duodenal ulcer is indicated if:

A. Repeated ulcerative attacks more than twice per year
B. Repeated vomiting of partially digested food
C. Single attack of melena in a young patient
D. Failure to respond to medical treatment for three weeks

Group AO

Regarding complications of gastroesophageal reflux, one of the following is FALSE:

A. Squamous cell carcinoma
B. Esophageal stricture
C. Shortening of esophagus
D. Metaplasia & dysplasia of lower esophageal mucosa

Complications of anti-reflux surgery for GERD include all the following EXCEPT:

A. Gas bloat syndrome
B. Pneumothorax
C. Dumping syndrome
D. Recurrent reflux
E. Dysphagia

64-year-old diabetic on omeprazole, chronic oily diarrhea, macrocytic anemia, mild hypoalbuminemia. Most likely diagnosis?

A. Hemochromatosis
B. Crohn's disease
C. Bacterial overgrowth syndrome
D. Intestinal lymphoma
E. Chronic pancreatitis

Gastrin-secreting G cells in the stomach are located in the:

A. Fundus
B. Pyloric canal only
C. Body
D. Pyloric antrum

Group AP

All the following about linitis plastica are true EXCEPT:

A. The stomach is stiff (leather-bottle appearance)
B. The stomach is narrowed
C. The prognosis is poor
D. The tumor may not be visible on the mucosa
E. It is a lymphoma of the stomach

Which of the following is NOT a risk factor for the development of esophageal malignancy?

A. Excessive intake of alcoholic spirits
B. Esophageal metaplasia
C. Smoking
D. Blood group O

All are useful to prevent PUD recurrence EXCEPT:

A. Avoid frequent small meals
B. Stop smoking
C. Avoid alcohol
D. Avoid NSAIDs

Regarding Barrett's esophagus:

A. Occurs in 40% of cases of GERD
B. Regular endoscopic surveillance is mandatory
C. Best treated with proton pump inhibitors
D. Predisposes to squamous cell carcinoma
E. It is a squamous metaplasia of the lower third

In bleeding per rectum, which is TRUE?

A. Meckel's is the commonest cause in adults >60 y
B. Massive bleeding never occurs in diverticular disease
C. Commonest cause is anal lesions e.g. hemorrhoids
D. Angiodysplasia usually affects the left side of the colon

Group AQ

In malignant tumors of the esophagus, all are true EXCEPT:

A. Are commonest at the upper third
B. Occur most commonly in males
C. Spread commonly by lymphatics
D. Are mostly squamous cell carcinoma

Which of the following conditions are precancerous?

A. Corrosive stricture
B. Barrett's esophagus
C. Achalasia
D. All of the above

The following features characterize UC EXCEPT:

A. Crypt abscess and cryptitis
B. Pseudopolyps
C. Superficial mucosal ulceration
D. Stricture formation in chronic cases

Group AR

About complications of chronic peptic ulcers, all are true EXCEPT:

A. Posterior duodenal ulcers tend to cause substantial bleeding
B. Benign gastric ulcers have a 50% tendency to malignant transformation
C. Anterior duodenal ulcers tend to perforate
D. The primary treatment of bleeding ulcers is usually non-surgical
E. Fibrosis of a chronic duodenal ulcer can cause pyloric obstruction

---

Which is the definitive treatment for a patient with actively bleeding esophageal varices?

A. Blood transfusion
B. Endoscopic band ligation
C. Balloon tamponade
D. Vasopressin

In perforated duodenal ulcer, all are true EXCEPT:

A. Produces abdominal tenderness most marked in the epigastrium
B. Usually accompanied by leukocytosis
C. Occurs most frequently in women
D. Usually preceded by exacerbation of ulcer symptoms

About acute gastric dilatation, TRUE statement:

A. Treated by gastrectomy
B. Hypovolemia is prominent
C. Rare postoperative complication
D. Causes hyperkalemia
E. Stomach shows hyperperistalsis

Group AS

During exploration for acute abdomen a hole in the anterior wall of the duodenum with turbid bile is found. Surgical steps include all EXCEPT:

A. Sub-hepatic, sub-diaphragmatic and pelvic drains
B. Thorough peritoneal lavage
C. Vagotomy and gastric drainage in the acute setting
D. Omental patch over the hole (Graham patch)

In Ulcerative colitis all are true EXCEPT:

A. Runs a steady progressive course with no remission
B. Presents with frequent watery blood-stained diarrhea
C. Incidence of malignancy is proportional to extent of colonic involvement
D. Toxic megacolon should be treated with surgical resection

As regards staging of cancer stomach, which is TRUE?

A. Invasion of up to 7 regional L.N. is considered N3
B. Metastases to bone and liver are of equal incidence in advanced stage
C. Serosa is invaded only in T4
D. Enlarged supraclavicular L.N. is evident in advanced cases

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A 47-year-old man presents with several hours of severe abdominal pain, board-like rigidity, shifting dullness and loss of liver dullness, after NSAID intake for backache. Best management is:

A. IV ranitidine and observation
B. Immediate surgery (after resuscitation)
C. Ryle tube insertion only
D. Immediate blood transfusion

The least common complication of ACUTE peptic ulcer disease is:

A. Pyloric stenosis
B. Microcytic hypochromic anemia
C. Hemorrhage
D. Perforation

Group AT

A 50-year-old man with 2-year dyspepsia has a small pyloric gastric ulcer on endoscopy. Biopsy shows chronic inflammation, and the ulcer is not responding to PPI. Most likely diagnosis:

A. Intestinal hyperplasia of the ulcer mucosa
B. Inherited APC gene mutation
C. Helicobacter pylori infection
D. Vitamin B12 deficiency

Regarding corrosive esophageal injury:

A. Early corticosteroids are recommended
B. Acids cause more injuries than alkalies
C. Perforation is not a complication
D. Endoscopic dilatation should be started immediately

For early carcinoma of the lower third of the esophagus, the best hope for cure is:

A. Gastrostomy
B. Radiotherapy
C. Chemotherapy
D. Esophagectomy and gastric pull-up
E. Endoscopic stenting

Group AU

Hematogenous spread of colonic adenocarcinoma is primarily to:

A. Lungs
B. Brain
C. Bones
D. Liver

Cell of origin of GIST:

A. G cells
B. Peptic cells
C. Cells of Cajal
D. Mucus cells
E. Parietal cells

Which structure does NOT pass through one of the three main diaphragmatic openings?

A. Left gastric artery
B. Esophagus
C. Aorta
D. Inferior vena cava
E. Left phrenic nerve

Regarding gastrointestinal bleeding, one of the following is FALSE:

A. Hematemesis arises distal to the ligament of Treitz
B. Melena usually signifies upper GI bleeding
C. Endoscopy is the main investigation for upper GI bleeding
D. Hematemesis is due to bleeding proximal to the ligament of Treitz

Group AV

5-year survival after esophagectomy for EARLY esophageal cancer:

A. 80–90%
B. 60–70%
C. <40%
D. 70–80%
E. >90%

The carcinoma commonly arising in Barrett's esophagus is:

A. Squamous cell carcinoma
B. Adenocarcinoma
C. Adenosquamous carcinoma
D. Transitional cell carcinoma

All the following about pyloric obstruction in adults are true EXCEPT:

A. Upper GI endoscopy is indicated
B. Vomiting is characteristically bilious
C. May cause tetany
D. May cause metabolic alkalosis

In achalasia of the cardia, all are true EXCEPT:

A. There is impaired peristalsis at the lower esophageal sphincter
B. May be complicated by esophageal carcinoma
C. Characterized by 'bird's beak' appearance on contrast radiography
D. Dysphagia is equal for fluids and solids

All increase risk of gastric cancer EXCEPT:

A. Smoking
B. H. pylori infection
C. Pernicious anemia
D. Prolonged H2 blocker intake
E. Distal gastrectomy (stump)

Group AW

Complications of duodenal ulcer include:

A. Hour-glass stomach
B. Malignant transformation
C. Gastric outlet obstruction
D. Obstructive jaundice

About gastric lymphoma, all TRUE EXCEPT:

A. Better prognosis than adenocarcinoma
B. Chemotherapy improves outcome
C. Surgery when possible gives best results
D. Usually Hodgkin type
E. Less frequent than adenocarcinoma

Boerhaave's syndrome refers to:

A. Mucosal tear at GE junction after forceful vomiting
B. Iatrogenic perforation during endoscopy
C. Corrosive-induced perforation
D. Full-thickness spontaneous esophageal perforation after forceful vomiting

Group AX

Transcoelomic spread of gastric carcinoma goes mainly to:

A. Liver
B. Ovary (Krukenberg)
C. Intestine
D. Kidney

67-year-old with dysphagia + hoarseness. Endoscopy: middle 1/3 esophageal cancer + right vocal cord paralysis. Best management:

A. Tracheostomy
B. Esophagectomy through right 5th ICS
C. Esophagectomy through left thoraco-abdominal incision
D. Esophageal stent

All the following weaken the gastric mucosal barrier EXCEPT:

A. Prostaglandin analogues
B. Duodeno-gastric reflux
C. H. pylori
D. Smoking
E. Shock

Regarding gastric peptic ulcer classification (Modified Johnson), which is FALSE?

A. Type I ulcers occur in the lesser curve with normal or subnormal acid secretion
B. Type II ulcers are located in the antrum only
C. Malignant transformation is rare
D. Type II is usually associated with duodenal ulcer

Group AY

Complications of gastroesophageal reflux include:

A. Barrett's esophagus
B. Stricture
C. Reflux esophagitis
D. Hematemesis
E. All the above

Which test is usually NORMAL in bacterial overgrowth syndrome?

A. D-Xylose absorption
B. Stage II Schilling test
C. Fecal fat (24h)
D. Jejunal aspirate culture
E. Lactulose breath test

Disaccharidase deficiency is an example of:

A. Exudative diarrhea
B. Osmotic diarrhea
C. Motility diarrhea
D. Secretory diarrhea

Regarding gastric carcinoma, all are true EXCEPT:

A. Rarely affects the antrum of the stomach
B. Troisier's sign carries a bad prognosis
C. May complicate pernicious anemia
D. May be associated with dietary nitrosamines and smoked foods

---

A recent Japanese immigrant with 3-month indigestion + gastric mass. Most likely diagnosis:

A. Chronic peptic ulcer
B. Multiple acute stress ulcers
C. Peutz-Jeghers syndrome
D. Gastric adenocarcinoma
E. Chronic gastritis

Group AZ

Which is NOT a predisposing factor for gastric cancer?

A. Foods with preservatives
B. Gastric polyp
C. H. pylori
D. Biliary gastritis
E. Iron deficiency anemia

What differentiates malabsorption in tropical sprue vs bacterial overgrowth?

A. Serum calcium
B. Serum folate
C. Serum B12
D. Serum iron

All are frequent complications of chronic PUD EXCEPT:

A. Carcinomatous change (only in gastric, very rare in duodenal)
B. Bleeding
C. Scarring deformity (stenosis)
D. Perforation

Group BA

Microbiology

Salmonella spp. have the following biochemical reactions EXCEPT:

A. Ferment lactose
B. Urease negative
C. Indole negative
D. Ferment glucose

In a retirement-home outbreak, what is the primary reservoir of infection?

A. Birds
B. Cattle
C. Rodents
D. Human

Which culture medium is used for curved gram-negative bacilli growing at 42°C?

A. Thayer-Martin medium
B. MacConkey agar
C. Skirrow's media
D. Lowenstein-Jensen medium

Group BB

Microbiology

Which organism would NOT benefit from prompt antibiotic treatment?

A. C. botulinum
B. C. perfringens
C. B. anthracis
D. C. difficile

Bacillus cereus is the etiological agent of all of the following EXCEPT:

A. "Long-incubation" or diarrheal form of food poisoning
B. Non bloody diarrhea
C. Cutaneous anthrax
D. "Short-incubation" or emetic form of food poisoning

Which stool study is specifically recommended for hospital-acquired diarrhea or diarrhea after recent antibiotics?

A. Fecal occult blood only
B. Ova and parasites only
C. Clostridioides difficile toxin testing
D. Stool elastase

Rodents are reservoirs of Leptospira which colonize their:

A. Eye
B. Skin
C. Spleen
D. Kidneys

Group BC

Microbiology

Salmonella causing food poisoning include:

A. Salmonella typhi
B. Salmonella typhimurium
C. Salmonella paratyphi A
D. Salmonella paratyphi B

The laboratory test of choice to diagnose brucellosis is:

A. Gram stain
B. Direct culture on nutrient agar
C. Blood film
D. Blood culture

A 32-year-old has had diarrhea for two weeks, sometimes with dysentery, after taking antibiotics for sinusitis before symptoms began. Which organism is most likely?

A. Clostridium difficile
B. Staphylococcus aureus
C. Salmonella typhi
D. Campylobacter jejuni

Culture of Campylobacter is done under the following condition:

A. Strict anaerobe
B. Micro-aerophilic and capnophilic
C. Strict aerobe
D. Facultative anaerobe

An extracellular virulence factor of Campylobacter is:

A. Flagella
B. Adhesins
C. Enterotoxins (LT)
D. Endotoxin (LPS)

Group BD

Parasitology

> What is the most common infectious cause of chronic diarrhea?

A. Shigella dysenteriae
B. Salmonella enterica
C. Giardia lamblia
D. Norovirus

> Which snail is the intermediate host of Schistosoma japonicum in the GI book's table?

A. Lymnaea species
B. Biomphalaria alexandrina
C. No snail host
D. Onchomelania species

> Which Schistosoma life-cycle stage hatches from eggs in fresh water and penetrates the snail?

A. Cercaria
B. Miracidium
C. Adult female
D. Schistosomulum

Group BE

Parasitology

What is the mode of infection of Taenia saginata?

A. Ingestion of undercooked meat
B. Ingestion of contaminated water
C. Ingestion of raw pork
D. Penetration of larvae through the skin

What is the diagnostic stage of Schistosoma mansoni detected in stool?

A. Eggs
B. Adult worms
C. Miracidia
D. Cercariae

Cysticercus bovis is the infective stage of which parasite?

A. Taenia saginata
B. Echinococcus granulosus
C. Diphyllobothrium latum
D. Taenia solium

Which stage of Schistosoma infects humans after exposure to contaminated fresh water?

A. Egg
B. Cercaria
C. Miracidium
D. Adult worm

Group BF

Parasitology

A 12-year-old Egyptian boy with a history of swimming in irrigation canals presents with blood and mucus in the stool. Stool examination reveals parasitic eggs. What is the treatment?

A. Albendazole
B. Praziquantel
C. Niclosamide
D. Mebendazole

Where do adult Schistosoma haematobium worms reside?

A. Vesical venous plexus
B. Inferior mesenteric venous plexus
C. Pulmonary arteries
D. Bile ducts

What is the intermediate host of Schistosoma mansoni in the GI book's life-cycle table?

A. Mosquitoes
B. Humans
C. Biomphalaria alexandrina
D. Onchomelania species

The diagnostic stage of Ascaris lumbricoides is:

A. Cercaria
B. Encysted metacercaria
C. Egg (in stool)
D. Egg and cercaria

Group BG

Pharmacology

What is the mechanism of action of vonoprazan?

A. Irreversibly inhibits the proton pump
B. Inhibits gastric prostaglandin synthesis
C. Reversibly blocks proton pump potassium-binding site
D. Competitively blocks H2 receptors

Which combination targets the underlying cause of an H. pylori-associated ulcer?

A. A laxative with a diuretic
B. A proton pump inhibitor with two or more antibiotics
C. Acid suppression alone
D. Antacids alone

Which drug most commonly causes upper GI bleeding?

A. Calcium carbonate
B. Paracetamol
C. Famotidine
D. Aspirin

Group BH

Pharmacology

When is the best time to take a PPI for GERD?

A. Immediately after the first meal
B. At bedtime after dinner
C. Only when symptoms begin
D. 30–60 minutes before first meal

How do NSAIDs cause gastric ulcers?

A. By blocking histamine H2 receptors
B. Inhibition of prostaglandin synthesis
C. By increasing gastric mucus production
D. By increasing prostaglandin synthesis

What is the mechanism of action of omeprazole?

A. Reversibly blocks the proton pump potassium-binding site
B. Irreversibly inhibits H^+/K^+ ATPase in parietal cells
C. Blocks muscarinic receptors on parietal cells
D. Competitively blocks H2 receptors

What is the drug of choice for moderate to severe GERD?

A. Sucralfate alone
B. Proton pump inhibitors
C. Antacids alone
D. Antispasmodics

Group BI

Pharmacology

What is an eradication therapy for H. pylori?

A. Triple therapy (PPI + amoxicillin + clarithromycin)
B. Antacid and sucralfate
C. PPI alone
D. Amoxicillin and metronidazole without acid suppression

How can peppermint oil help some patients with IBS according to the GI book?

A. It relaxes GI smooth muscle
B. It acts as a bile acid
C. It increases gastric acid secretion
D. It eradicates H. pylori

Which treatment listed in the GI book is directed at IBS-D?

A. Lubiprostone
B. Mesalazine
C. Pyloroplasty
D. Rifaximin

Which medication group may help relieve IBS-associated abdominal pain?

A. Proton pump inhibitors for every patient
B. Tricyclic antidepressants
C. Loop diuretics
D. Oral anticoagulants

Which drug class is represented by lubiprostone in IBS-C treatment?

A. Chloride channel activator
B. Proton pump inhibitor
C. Bile acid sequestrant
D. H2 receptor blocker

Group BJ

Pharmacology

Which medication group can contribute to functional acute gastric dilatation by suppressing GI motility?

A. Bile acid binders
B. Oral iron only
C. High-dose anticholinergics
D. Prokinetic agents

Which of the following is NOT true concerning bismuth subsalicylate?

A. It should be given for more than 4 months
B. It is used in quadruple anti-H. pylori regimen
C. It acts as mucosal protective agent
D. It may cause neurotoxicity & encephalopathy

Which of the following phases of acid secretion is most likely affected by H2 receptor antagonists?

A. Gastrin stimulated acid secretion
B. Both nocturnal and meal stimulated phases
C. Nocturnal acid secretion
D. Meal stimulated acid secretion

Group BK

Pharmacology

Which of the following is NOT correct concerning misoprostol?

A. Contraindicated during pregnancy as it may induce abortion
B. A cytoprotective agent used in all types of peptic ulcer if H2 blockers fail
C. Diarrhea and abdominal cramps are its most common adverse effects
D. PGE analog that increases mucosal blood flow, mucus & bicarbonate

Which of the following medications is mostly included in anti-H. pylori regimens?

A. Doxycycline
B. Misoprostol
C. Clarithromycin
D. Bismuth subcitrate

Which of the following is a key advantage of P-CABs compared to proton pump inhibitors (PPIs)?

A. P-CABs require less frequent dosing than PPIs
B. P-CABs are more effective antisecretory drugs regardless of food intake
C. P-CABs are less effective in treating Helicobacter pylori infections
D. P-CABs have a longer duration of action

Which of the following mucosal protective agents has anti-H. pylori effect?

A. Sucralfate
B. Misoprostol
C. Bismuth subcitrate
D. Bicarbonate

Group BL

Pharmacology

Systemic alkalosis could be an adverse effect of long term use of which of the following antacids?

A. Aluminum hydroxide
B. Magnesium oxide
C. Magnesium hydroxide
D. Sodium bicarbonate

A 45-year-old woman with peptic ulcer disease is treated with misoprostol. Which of the following is a common adverse effect?

A. Hypertension
B. Diarrhea
C. Constipation
D. Headache

Which of the following is correct concerning pharmacokinetics of H2-receptor antagonists?

A. 50% of the dose of ranitidine is destroyed by gastric acidity
B. They are contraindicated in mild renal dysfunction due to renal excretion
C. 50% of the dose of famotidine is lost by hepatic first pass effect
D. Most of them have low to moderate bioavailability except nizatidine

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A 50-year-old woman with GERD is prescribed famotidine. Which is the primary mechanism of action of famotidine?

A. Blocking histamine H2 receptors in the stomach
B. Inhibition of the proton pump
C. Blocking muscarinic receptors on parietal cells
D. Neutralization of gastric acid

Which of the following antacids has the most rapid and potent effect in hyperacidity?

A. Sodium bicarbonate
B. Magnesium oxide
C. Aluminum hydroxide
D. Magnesium hydroxide

Group BM

Pharmacology

Which of the following antacids is mostly preferred for hyperacidity?

A. Both Mg and Al salts together
B. Sodium bicarbonate
C. Aluminum hydroxide
D. Magnesium hydroxide

NSAIDs cause acute gastritis through:

A. Decreased PGE2
B. Increased histamine
C. Decreased CCK
D. Increased PGE2

Which drug increases the risk of GERD?

A. High-protein diet
B. Baclofen
C. β-blockers
D. Calcium channel blockers